



ICD-0-3

Carcinoma, Urothelial 8120/3

Path Site: Bladder; lateral, anterior
and posterior walls C69.8

C69.4
C69.4 Site: Bladder; Posterior Wall

Gender: Female

Race: White

Pathology Report:

QW 12/24/12

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Right pelvic lymph nodes; dissection:

- Eighteen lymph nodes, no tumor (0/18).

B. Left pelvic lymph nodes; dissection:

- Twelve lymph nodes, no tumor (0/12).

C. Urinary bladder; cystectomy:

- Invasive high grade urothelial carcinoma, invading thru the muscularis propria and into the perivesical tissue, see pathologic parameters.

D. Uterus and round ligaments; supracervical hysterectomy:

- Uterus with inactive endometrium and adenomyosis.
- Bilateral fallopian tubes, no tumor.
- Bilateral ovaries with age-related physiologic changes, no tumor.

E. Distal right ureter; excision:

- Portion of ureter, no tumor.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma.
2. Grade of tumor: High grade.
3. Depth of invasion: Extravesicular soft tissue (macroscopic).
4. Tumor distribution: Solitary, 1.5 cm with ulceration, right lateral, anterior and posterior walls.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.
8. Lymph nodes: Negative for tumor (0/30).
9. pTNM: pT3b,N0,MX.

^ this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

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Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], M.D.

Electronically Signed Out by [] MD

Clinical History:

Patient is a year-old female with high grade urothelial carcinoma invasive to the muscularis propria on biopsy undergoing cystectomy, hysterectomy, and bilateral oophorectomy.

Specimens Received:

A: Right pelvic lymph node

B: Left pelvic lymph node

C: Bladder

D: Uterus and round ligaments

E: Distal right ureter

Gross Description:

The specimens are received in five containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "right pelvic lymph node". Received fresh and placed in formalin is a 9 x 6 x 2 cm aggregate of yellow-tan, lobulated adipose tissue. This specimen is dissected for lymph nodes and 23 her found ranging from 0.3-3.7 cm in greatest dimension. The lymph nodes are submitted entirely as follows:

A1: 6 lymph node candidates

A2: 6 lymph node candidates

A3: 6 lymph node candidates

A4-A6: One lymph node, bisected in each cassette (3 total)

A7-A8: One lymph node, bisected

A9-A10: One lymph node, sectioned

B. The second container is additionally identified as, "left pelvic lymph nodes". Received fresh and placed in formalin is a 7 x 4.5 x 1.5 cm aggregate of yellow-tan, lobulated adipose tissue. The specimen is dissected for lymph nodes and 14 are found ranging in size from 0.5-5.5 cm in greatest dimension. The lymph nodes are submitted entirely as follows:

B1: 6 lymph node candidates
B2: 6 lymph node candidates
B3-B4: 1 lymph node, bisected
B5-B7: One lymph node, sectioned

C. The third container is additionally identified as, "bladder". Received fresh and placed in formalin is a 312 g cystectomy specimen with overall measurements of 15 x 12 x 4 cm. The right ureteral stump measures 2.5 x 0.4 cm, and the left measures 2.5 x 0.4 cm, and both demonstrate intact patent lumens. The serosal surface shows yellow lobulated fibroadipose tissue with focal cautery artifact. The urethral margin measures 2.3 x 1.5 cm, is inked black, and the bladder is inked black. The specimen is opened anteriorly to reveal a 4.5 x 4.5 cm indurated, ulcerated lesion in the right lateral/anterior/posterior wall. On cut section, the mass extends into the perivesicular soft tissue and is 0.3 cm from the closest deep margin. There is a 0.5 cm firm, white nodule in the perivesicular adipose tissue 0.3 cm deep to the bladder mass and 9 cm from the inked margin. In the right distal bladder there is a 1.5 x 1.2 x 0.7 cm firm tan-white, ill-defined area in the perivesicular soft tissue with punctate hemorrhage and no corresponding mucosal lesion. The surrounding bladder mucosa is edematous, pink-tan, smooth with a 0.5-2.7 cm wall thickness. Bilateral ureteral orifices, adjacent to the trigone, are identified and probe patent.

Representative sections are submitted as follows:

C1: Distal urethral margin
C2: Right and left ureter resection margin (right side is inked blue)
C3-C4: Bladder mass with invasion to prevesicular tissue and nodule (one full thickness section, bisected)
C5: Bladder mass closest to ink and adjacent grossly unremarkable mucosa
C6: Bladder mass and adjacent grossly unremarkable mucosa
C7: Additional section of mass
C8: Uninvolved the bladder dome
C9: Uninvolved posterior left
C10: Right distal perivesicular firm tissue

D. The fourth container is additionally identified as, "uterus and round ligament". Received fresh and placed in formalin is 49 gm supracervical hysterectomy specimen measuring 3.8 cm from fundus to lower uterine segment by 3.8 cm from cornu to cornu by 2.5 cm from anterior to posterior. The serosal surface is red-tan, pink-tan M. roughened. The specimen is opened revealing a triangular endometrial cavity measuring 2.5 cm from cornu to cornu by 2.8 cm in length and lined with grossly atrophic endometrium measuring 0.1 cm in thickness. The specimen is sectioned reveal pink-tan, trabeculated myometrium which is up to 1.1 cm in greatest thickness.

The right adnexa structure consists of a 7.5 cm long and 0.4 cm diameter fimbriated fallopian tube. The serosa is pink-red and glistening. The specimen

is serially sectioned revealing a pinpoint lumen and less than 0.1-0.2 cm wall thickness. The attached white-tan, cribriform ovary measures 3.5 x 1.5 x 1 cm and is sectioned to revealed fibrotic, white-tan ovarian stroma with multiple corpora lutea.

The left adnexa structure consists of a 6.5 cm long and 0.5 cm diameter fimbriated fallopian tube. The serosa is pink-red and glistening. The specimen is serially sectioned revealing a pinpoint lumen and a less than 0.1-0.2 cm wall thickness. The attached white-tan, cribriform ovary measures 2.5 x 1.3 x 0.8 cm and is sectioned to revealed fibrotic, white-tan ovarian stroma with multiple corpora lutea.

Also received in the same container is a separate portion of brown ligament and adipose tissue measuring 3.3 x 1.2 x 0.5 cm.

Representative sections are submitted as follows:

- D1: Anterior endomyometrium
- D2: Posterior endomyometrium
- D3: Sections of right fallopian tube
- D4: Right ovary
- D5: Sections of left fallopian tube
- D6 left ovary
- D7: Round ligament

E. The fifth container is additionally identified as, "distal right ureter stitch on the non-margin".

Received fresh and placed in formalin is a 1.5 cm long segment of appearing ureter with attached adipose tissue. There is a clear suture designating the non-margin side. The margin side is inked blue, the specimen is bisected and submitted as E1.

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[], M.D.

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
In PAA Discrepancy		✓
or for Malignancy History		✓
Discrepancy/Primary Noted		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	km1	12/18/12